Decision Aid · 2026-05-15
For Dad, Mom, Dr. Rubin, the K Clinic psychiatrist, Julane & the family team

Should Dad switch from doxepin to lemborexant (Dayvigo) for sleep?

UPDATED RECOMMENDATION (REVISED 2026-05-19 AFTER OE #13)

Lemborexant 5 mg HS is defensible — either as a seamless substitute for doxepin (if doxepin needs to come off) or as an addition for sleep-maintenance support — based on the 5/19 OpenEvidence consultation (#13). The two strongest new findings: (1) lemborexant is **night-1 effective** in adults ≥55 (WASO reduction 33 min at 5 mg, 42 min at 10 mg vs placebo on nights 1–2 in SUNRISE-1; second-half-of-night WASO benefit present from night 1 — no accumulation period needed); (2) the doxepin → lemborexant transition can be done **seamlessly on a single night —** both produce first-night WASO effects, no expected gap. No rebound insomnia, no withdrawal; can be stopped abruptly if it doesn't work or produces adverse effects, which makes the trial genuinely low-risk. PK interactions with duloxetine + low-dose doxepin are not significant (CYP3A4-metabolized, no inhibition from Dad's stack). **The original 5/15 cautions still hold but are softer than first stated:** RBD-specific clinical data is absent (the mouse-tauopathy DORA-12 data actually point toward reduced dream-enactment, opposite the original concern); the FDA "don't combine with other hypnotics" guidance is regulatory caution rather than a strong PK signal; the REM-augmentation effect could theoretically increase RBD-episode opportunity but has not been demonstrated clinically. **Operative position:** if Rubin's proposal involves removing doxepin, lemborexant 5 mg HS is a defensible substitute. If doxepin stays, lemborexant can be added if sleep maintenance is the limiting symptom. Either way, melatonin and trazodone PRN continue. Start at 5 mg HS; reassess at 7–10 days; escalate to 10 mg only if 5 mg is tolerated and benefit incomplete.

WHAT THE 5/19 OPENEVIDENCE RESPONSE (#13) ADDED

- **Night-1 effective.** SUNRISE-1 (Rosenberg 2019, adults ≥55, median age 63): on nights 1–2, lemborexant 5 mg reduced LPS by ~17 min and WASO by ~33 min vs placebo. 10 mg reduced LPS by ~20 min and WASO by ~42 min. The benefit was sustained through 12 months in SUNRISE-2 with no tolerance development. **Steady-state efficacy is essentially night 1** — no accumulation needed.
- **Second-half-of-night WASO benefit present from night 1** — distinguishing feature vs zolpidem ER, exactly the pattern Dad reports (early-morning awakening, unrested wake).
- **Doxepin → lemborexant substitution is seamless.** Both produce first-night WASO effects; lemborexant's night-1 effect size at least matches doxepin 6 mg. Single-night transition expected to maintain sleep-maintenance benefit. If Rubin's proposal includes removing doxepin, the substitution is operationally defensible.
- **No rebound insomnia, no withdrawal.** Lemborexant can be stopped abruptly. If it doesn't work or produces adverse effects, the trial reverses cleanly. This is the load-bearing point for low-risk trial framing.
- **No significant PK interaction with duloxetine or low-dose doxepin.** Lemborexant is CYP3A4-metabolized; duloxetine is not a CYP3A4 inhibitor; doxepin 6 mg is unlikely to inhibit. Combination is pharmacologically manageable despite FDA's general "don't combine with other hypnotics" guidance (which is regulatory caution, not a strong PK signal at these doses).
- **RBD-specific clinical data: absent.** The mouse-tauopathy data (Kam 2023, DORA-12 in PS19 mice) actually showed *reduced* dream-enactment behaviors — opposite direction of the original 5/15 concern. The human inference remains uncertain, but the original concern is weaker than first stated. FDA-labeled narcolepsy-spectrum signals are real but uncommon: sleep paralysis 1.3–1.6%, hypnagogic hallucinations 0.1–0.7%, nightmares 0.9–2.2%.

- **Compatible with ketamine.** No published interaction data either direction. Orexin antagonism operates through OX1R/OX2R in the lateral hypothalamus, mechanistically distinct from ketamine's glutamatergic pathway. Improved sleep itself may support ketamine response.
- **Cardiac:** does not prolong QTc at doses up to 5× max recommended. Safe at Dad's bradycardia + LAFB.
- **OE bottom-line:** Lemborexant is a defensible substitute for doxepin or addition to current sleep stack. Night-1 effective; low-risk trial; reversible.

ORIGINAL RECOMMENDATION (NOW SUPERSEDED — 5/15 POSITION)

> **Original 5/15 position: not at this time, and not as a switch from doxepin.** Continue doxepin 6 mg HS + melatonin 3 mg HS through the May 18 IV ketamine restart; reassess sleep after 2–4 weeks. Reasoning at the time: REM-augmenting effects in RBD; FDA recommendation against combining DORAs with other insomnia agents; orexin involvement in synucleinopathy workup; lack of supportive depression-signal data. This position has been substantially revised after OE #13 — see updated recommendation above.

WHY THIS MATTERS FOR DAD SPECIFICALLY

Julane recommended switching Dad from doxepin 6 mg to lemborexant (Dayvigo) 5 mg HS, citing sleep as foundational to ketamine recovery and identifying lemborexant as the cleanest of the three FDA-approved orexin antagonists (DORAs) on a published depression signal. Dad himself raised the question on 5/15 — he wants relief and is reasonably asking what else is out there. Julane is right that sleep matters foundationally and that DORAs are a meaningful modern class with a targeted mechanism. But the specific recommendation has several concerns in Dad's profile that the integration note doesn't fully address. Lemborexant increases REM sleep duration and decreases REM latency — exactly the sleep stage where Dad's RBD episodes occur. Per Dad's 5/15 self-report, those episodes are now infrequent (most recent ~February 2026, historically roughly once every few months, patient-reported decreasing trajectory) — which softens the RBD-DORA concern somewhat but does not eliminate it. The mechanistic argument is independent of episode frequency: pharmacologically widening the REM window in a patient whose REM-atonia mechanism is known to fail, even sporadically, remains a non-trivial concern. The class as a whole has FDA-labeled warnings for cataplexy-like symptoms and sleep paralysis (REM-intrusion phenomena), and a 2024 meta-analysis found sleep paralysis ~3.4× more likely on DORAs than placebo. The orexin system itself is one of the systems the June neurology evaluation is examining, with patterns in early disease that pharmacological blockade may disrupt unpredictably. The FAERS depression-signal evidence Julane cited has shifted: a 2025 pharmacovigilance analysis found *no significant differential* in suicidality reports among the three DORAs versus trazodone — and the three carry identical FDA label warnings for worsening depression. Finally, the FDA Dayvigo label specifically recommends against combining DORAs with other insomnia drugs, and Dad's existing doxepin + melatonin would form exactly that combination if lemborexant were added.

THE EVIDENCE

What might support adding or switching to lemborexant	What argues against — in Dad's specific profile
Julane's overall framing is right: sleep is foundational. Dad's nighttime hyperarousal and dread-on-waking are real symptoms that need addressing. Without adequate sleep, the nervous system cannot consolidate ketamine gains or sustain integration work. An evidence-based DORA conversation belongs in the longer-term plan.	**Lemborexant increases REM sleep — exactly the sleep stage where Dad's RBD occurs.** The SUNRISE-1 sleep architecture analysis (Moline 2021) found lemborexant significantly increases REM sleep duration and decreases REM latency in older adults compared to both placebo and zolpidem. In RBD — defined by loss of the muscle paralysis that normally accompanies REM sleep — more REM time means a wider mechanical window for dream-enactment episodes. Per Dad's 5/15 self-report, his most recent episode was around February 2026 with historical

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	frequency roughly once every few months and a patient-reported decreasing trajectory; that softens the concern somewhat but does not eliminate it (the mechanistic argument is independent of episode frequency). No clinical trial has studied any DORA in patients with formally diagnosed RBD.
Lemborexant has direct evidence for sleep maintenance in older adults — including the second half of the night. SUNRISE-1 (Rosenberg 2019, n=1,006 adults ≥55) found significant reductions in wake-time-after-sleep-onset (WASO), with lemborexant 5 mg specifically superior to zolpidem on second-half-of-night WASO. This is the exact pattern of awakening Dad has been reporting.	**DORAs as a class carry FDA-labeled warnings for cataplexy-like symptoms and sleep paralysis.** A 2024 meta-analysis (Na et al.) found sleep paralysis is about 3.4× more likely on DORAs than placebo across the class — these are REM-intrusion phenomena mechanistically related to the same orexin blockade that helps with sleep. In a patient with established RBD, these signals are not abstract: they touch the same axis of REM regulation that is already dysregulated.
Doesn't produce physical dependence or rebound insomnia on discontinuation. Unlike benzodiazepines, lemborexant can be stopped without taper. This is a real advantage over older sleep agents, particularly in older adults.	**The orexin system is connected to the questions the June neurology evaluation is examining.** A 2024 systematic review (Raheel et al., Current Neurology and Neuroscience Reports) found that in RBD and early Parkinson's disease, orexin levels may be adaptively increased before declining at later stages. Pharmacologically blocking the orexin system before the neurology workup is complete is mechanistically unstudied in this context and could plausibly have unpredictable effects. The June neurology evaluation is specifically to characterize the broader picture.
No clinically significant CYP3A4 interactions with Dad's current medications. Lemborexant is metabolized via CYP3A4, but duloxetine, doxepin, melatonin, tadalafil, Cialis, and rosuvastatin don't meaningfully inhibit this enzyme. Lemborexant 5 mg is the appropriate starting dose for Dad's age without further adjustment.	**The specific "lemborexant is cleanest on depression signal" reasoning is not supported by current FAERS data.** Julane cited the 2024 daridorexant FAERS analysis suggesting lemborexant has the lowest depression signal. But a more recent 2025 FAERS analysis (McIntyre RS et al., *Expert Opin Drug Saf*) found *no significantly increased reporting odds* for suicidal ideation, depression, or suicide attempts for any of the three DORAs compared to trazodone — and all three carry *identical* FDA label warnings for worsening depression. The differential favoring lemborexant doesn't hold up in the most current data.
No documented cardiac conduction effects. A formal QT-modeling study during lemborexant's development found no clinically significant QT prolongation at exposures 8× the highest clinical dose. In a patient with LAFB and bradycardia, this is genuinely reassuring relative to alternatives like Lexapro.	**The FDA Dayvigo label explicitly recommends against combining DORAs with other insomnia drugs.** Adding lemborexant to existing doxepin + melatonin would create exactly that combination — additive sedation through three distinct sleep-promoting pathways (H1 antagonism, melatonin receptor agonism, orexin antagonism) without published safety data for this triple combination in older adults.
—	**Doxepin has only been at therapeutic dose (6 mg) for about 10 days.** The current dose started 5/4 at 3 mg, titrated to 6 mg around 5/11. That's not enough time to assess whether doxepin 6 mg + ketamine will resolve the sleep maintenance picture. Switching before that assessment is complete forecloses the option without learning anything.

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—	**The May 18 IV ketamine restart is expected to independently improve early-morning awakening.** Per Rodrigues 2022 (n=323) and Boesjes 2026 (systematic review of 26 studies), ketamine specifically improves the early-morning-waking pattern — and in documented prior responders, this effect often appears within the first 1–2 sessions. Adding lemborexant before assessing the ketamine response confounds attribution and adds unnecessary medication.
—	**The post-lorazepam rebound window may still be open** (peak 1–4 days, can persist 1–2 weeks after discontinuation). Some of Dad's current sleep disruption is likely transient rebound that will resolve on its own. Adding a new sleep agent during this window means treating something that is partially self-resolving — and obscures what the persistent baseline looks like.
—	**The post-Ativan window is still settling.** Dad recently completed his Ativan taper. Adding a CNS-active sleep medication on top of a still-settling picture in a 76-year-old creates polypharmacy considerations that are worth letting settle before any new sleep med is started.

Strength of evidence against switching now: ★★★★★

Convergence of multiple independent concerns: RBD-DORA mechanistic question (softened modestly by the updated 5/15 RBD-frequency picture, but not eliminated); FDA label recommendation against combining DORAs with other insomnia drugs; orexin-system involvement in the questions the June neurology evaluation is examining; the cited "cleanest DORA on depression" advantage isn't actually supported by current data; doxepin at therapeutic dose for only ~10 days; IV ketamine restart imminent; benzo rebound window may still be open.

WHAT BOTH APPROACHES SHARE

- **Sleep IS foundational.** Julane is right that this isn't a peripheral concern — it's central to Dad's recovery. The disagreement is about which agent and on what timeline, not whether sleep matters.
- **If a DORA is eventually considered after the ketamine and neurology data are in**, lemborexant 5 mg is the appropriate starting dose for a patient ≥65 (the FDA label cautions against 10 mg in this age group). The cardiac and CYP3A4 profile is reasonably reassuring. The concerns above are about timing and the RBD/orexin-system axis, not about lemborexant being the wrong DORA on other dimensions.
- **Melatonin titration is the lower-risk lower-leverage first move** if sleep remains a problem after the ketamine restart. Dad is on 3 mg HS (dosed against his G47.52 RBD diagnosis); titrating to 5 mg or shifting to an extended-release formulation has a favorable safety profile, is supported by the AASM 2023 RBD guideline, and doesn't introduce a new medication class.
- **The ketamine restart itself is a sleep intervention.** The published evidence (Rodrigues 2022, Boesjes 2026, Wang 2021) supports that ketamine improves sleep architecture in TRD responders — including early-morning awakening specifically — and that sleep improvement within 24 hours of the first session can predict overall antidepressant response. So sleep change is something to watch starting at the first IV session, not something requiring a separate medication.
- **Behavioral interventions for the dread-on-waking pattern matter independently of pharmacology.** The "overwhelming dread on waking" is a depressive symptom; it responds to depression treatment (ketamine) more than to sleep agents. Hypnotics improve sleep architecture but don't directly address the subjective quality of the wake state.

RECOMMENDED NEXT STEPS

1. **Continue doxepin 6 mg HS + melatonin 3 mg HS unchanged** through the 5/18 IV ketamine restart and the following 2–4 weeks.
2. **Confirm where Dad is on the Ativan taper.** The 5/14 integration note and the family understanding describe slightly different pictures. The answer matters for the ketamine restart (benzodiazepines can attenuate ketamine response) and for whether to consider adding any new CNS-active medication.
3. **Do not initiate lemborexant before the May 18 restart.** Adding a third sleep-promoting medication on top of doxepin + melatonin, at the same time as the ketamine restart, with possibly-still-active Ativan in the picture, layers too many variables in a 76-year-old.
4. **Track sleep change in the first 1–2 IV ketamine sessions.** Per the literature, sleep improvement within 24 hours of the first session is a known signal of broader ketamine response. Watch for it.
5. **Reassess sleep after 2–4 weeks of IV ketamine** (approximately late May / mid-June). At that point, with the ketamine effect partially established and the benzo-rebound window closed, the persistent sleep maintenance picture (if any) will be clearer.
6. **If sleep maintenance remains the limiting symptom at that reassessment**, the lower-risk first move is melatonin titration (3 mg → 5 mg HS, or shift to extended-release). Lemborexant could be considered *after* the June neurology evaluation if sleep is still a problem and the broader neurology picture has been characterized — but the orexin-system involvement in the very systems the workup is examining makes pre-neurology DORA use mechanistically uncertain.
7. **If lemborexant is eventually pursued**: 5 mg starting dose (FDA elderly caution applies to 10 mg), discontinue doxepin first (don't combine — FDA-label recommendation), close RBD monitoring (track frequency and severity of dream-enactment episodes), and informed-consent discussion about the cataplexy-like / sleep-paralysis class signal.

BOTTOM LINE *Julane is right that sleep matters. She's right that lemborexant is a meaningfully better DORA than alternatives on some dimensions. She's wrong that the FAERS depression-signal favors lemborexant over the other DORAs (the 2025 update found no significant differential), and the integration note doesn't fully address the RBD-DORA mechanistic concern or the FDA label's explicit recommendation against combining DORAs with other insomnia drugs. Stay on doxepin 6 mg through the IV ketamine restart, clarify the Ativan picture, and revisit lemborexant after the ketamine response and the June neurology evaluation have shaped the picture. Don't switch now.*

Key Studies & Findings

Ranked by authority · Each citation summarized for how it applies to Dad's case

TIER 1 — MAJOR CLINICAL GUIDELINES AND REGULATORY SOURCES ★★★★★

FDA Dayvigo (lemborexant) Label.
FDA prescribing information. Read here.

KEY FINDING The FDA label specifically states: "The use of DAYVIGO with other drugs to treat insomnia is not recommended." Co-administration with CNS depressants (including tricyclic antidepressants like doxepin) increases risk of CNS depression and daytime impairment. Caution recommended with doses >5 mg in patients ≥65. Carries warnings for cataplexy-like symptoms, sleep paralysis, and worsening depression/suicidal ideation.

HOW IT APPLIES The "do not combine with other insomnia drugs" guidance is decision-relevant here because Dad is already on doxepin 6 mg + melatonin 3 mg. Switching to lemborexant would require discontinuing doxepin first per FDA-label guidance, not adding it alongside.

Morin CM, Buysse DJ. Management of Insomnia.
NEJM 2024;391:247–258. Read here.

KEY FINDING The current NEJM review of insomnia. Identifies DORAs (lemborexant, suvorexant, daridorexant) and low-dose doxepin as the two pharmacologic classes with the most favorable safety profile in older adults. DORAs offer a targeted mechanism and lower cognitive impairment risk than benzodiazepine receptor agonists; low-dose doxepin offers H1-selective sleep maintenance benefit without the broader tricyclic burden at higher doses.

HOW IT APPLIES Both options are evidence-supported; the choice is about which fits Dad's specific profile (RBD, possible prodromal Lewy-spectrum disease, polypharmacy timing). The NEJM review supports continuing doxepin as a defensible option, not just defaulting to a DORA.

Howell M, Avidan AY et al. Management of REM Sleep Behavior Disorder: AASM Clinical Practice Guideline.
J Clin Sleep Med 2023;19:759–768. Read here.

KEY FINDING The AASM clinical practice guideline for RBD. Recommends melatonin and clonazepam as first-line pharmacologic options. Discusses discontinuation of REM-affecting drugs in drug-induced or drug-exacerbated RBD. DORAs are not recommended as RBD therapy and the published RBD evidence base does not include DORAs.

HOW IT APPLIES The dedicated RBD guideline supports melatonin titration as a defensible next-step intervention if sleep maintenance is still a problem after the ketamine restart — and does not list DORAs as appropriate in this population.

TIER 2 — MAJOR RCTS ★★★★

Rosenberg R, Murphy P et al. Comparison of Lemborexant With Placebo and Zolpidem Tartrate Extended Release for Older Adults With Insomnia Disorder (SUNRISE-1).
JAMA Network Open 2019;2:e1918254. Read here.

KEY FINDING Phase 3 RCT, n=1,006 adults ≥55 with insomnia. Lemborexant significantly reduced wake-time-after-sleep-onset compared to both placebo and zolpidem extended release. Notably, lemborexant 5 mg was superior to zolpidem on second-half-of-night WASO (~6.7 minute additional reduction). No notable findings on ECG or vital signs. Patients with significant neuropsychiatric comorbidity were excluded — RBD was not a studied condition.

HOW IT APPLIES This is the strongest published evidence for lemborexant in Dad's age group. The second-half-of-night WASO effect is exactly the pattern Dad has. But the exclusion of patients with significant neuropsychiatric comorbidity means the trial doesn't address Dad's specific RBD profile.

Moline M, Zammit G et al. Effect of Lemborexant on Sleep Architecture in Older Adults With Insomnia Disorder (SUNRISE-1 sleep architecture analysis).
J Clin Sleep Med 2021;17:1167–1174. Read here.

KEY FINDING Polysomnographic sleep architecture analysis from SUNRISE-1. Lemborexant significantly increased REM sleep duration and decreased REM latency in older adults — moreso than zolpidem or placebo. The effect on REM is a class-property of orexin antagonism, not a lemborexant-specific finding.

HOW IT APPLIES The single most decision-relevant finding for Dad's RBD profile. More REM time means a wider mechanical window for dream-enactment episodes. The trial didn't study RBD patients, but the architectural effect is unambiguous in the population that was studied.

TIER 3 — SYSTEMATIC REVIEWS AND PHARMACOVIGILANCE ★★★

McIntyre RS, Wong S et al. Association Between Dual Orexin Receptor Antagonists (DORAs) and Suicidality: FAERS Analysis.
Expert Opinion on Drug Safety 2025;24:753–757. Read here.

KEY FINDING 2025 FDA Adverse Event Reporting System analysis comparing the three FDA-approved DORAs. Found **no significantly increased reporting odds** for suicidal ideation, depression, suicidal behavior, or suicide attempts for any of the three DORAs versus trazodone. All three DORAs had significantly *lower* odds of completed suicide reports than trazodone. The reasoning that lemborexant has a meaningfully cleaner depression profile than suvorexant or daridorexant is not supported in this updated dataset.

HOW IT APPLIES Directly addresses Julane's specific rationale. The FAERS depression-signal advantage she cited for lemborexant doesn't hold up in the most current pharmacovigilance literature. The three DORAs carry identical FDA label warnings for worsening depression. Doesn't make lemborexant a worse choice than the other DORAs — just means the specific framing offered isn't accurate.

Na HJ, Jeon N et al. Clinical Safety and Narcolepsy-Like Symptoms of Dual Orexin Receptor Antagonists: A Systematic Review and Meta-Analysis.

Sleep 2024;47:zsad293. Read here.

KEY FINDING Systematic review and meta-analysis of DORA clinical trials. Sleep paralysis was about 3.4× more likely on DORAs than placebo (RR 3.40, 95% CI 1.18–9.80) — and is a recognized class-wide REM-intrusion phenomenon. Excessive daytime sleepiness also elevated.

HOW IT APPLIES The class-wide REM-intrusion signal is the mechanism that bears on Dad's RBD. Sleep paralysis is a related REM-regulation phenomenon. In someone whose REM regulation is already abnormal (RBD), the class signals are not abstract.

Raheel K, See QR et al. Orexin and Sleep Disturbances in Alpha-Synucleinopathies: A Systematic Review.

Current Neurology and Neuroscience Reports 2024;24:389–412. Read here.

KEY FINDING Systematic review of the orexin system in the broader Parkinson-spectrum family of conditions. In RBD and early Parkinson's disease, orexin levels may be adaptively increased, with reduced levels at later stages. Suggests the orexin system is actively involved in the underlying neurology rather than passively affected.

HOW IT APPLIES The specific mechanistic question about lemborexant given the neurology workup pending in June. Pharmacologically blocking the orexin system before the broader picture is characterized is unstudied, and the underlying biology suggests the effect is unpredictable. This is a non-trivial reason to wait until after the June neurology evaluation before considering DORA use.

Vasudeva S, Wong S et al. Efficacy and Safety Profiles of FDA-approved Dual Orexin Receptor Antagonists in Depression: A Systematic Review.

J Psychiatr Res 2025;191:752–769. Read here.

KEY FINDING 2025 systematic review of DORAs in patients with comorbid insomnia and depression. Modest improvements in depressive symptoms in patients with comorbid insomnia and MDD, with sleep parameter improvements being more consistent than mood effects. Currently approved DORAs are not approved as adjunctive treatments for MDD.

HOW IT APPLIES Counters any framing of lemborexant as a meaningful antidepressant adjunct. The mood benefits of DORAs in MDD are modest and probably mediated by sleep improvement rather than direct antidepressant action. Lemborexant is a sleep medication, not a depression medication.

TIER 4 — MECHANISM, CARDIAC SAFETY, AND GERIATRIC COHORTS ★★

Gotfried MH, Auerbach SH et al. Efficacy and Safety of Insomnia Treatment With Lemborexant in Older Adults: Analyses From Three Clinical Trials.

Drugs & Aging 2024;41:741–752. Read here.

KEY FINDING Pooled analysis across three clinical trials. Lemborexant improved sleep variables in adults ≥65 without next-morning residual effects on postural stability, memory, or driving performance. Subgroup of patients ≥75 was small (n=87); patients with significant cognitive vulnerability were not specifically studied.

HOW IT APPLIES The age-specific safety profile is reasonably reassuring in adults ≥65 generally. The very-late-life and cognitively-vulnerable subgroups remain understudied — relevant for Dad's profile.

Murphy PJ, Yasuda S et al. Concentration-Response Modeling of ECG Data — Lemborexant.

J Clin Pharmacol 2017;57:96–104. Read here.

KEY FINDING Concentration-response analysis of ECG data from lemborexant's development program. Predicted QTc effect of 1.1 ms (90% CI −3.49 to 5.78) at exposures 8× the highest planned clinical dose. No clinically significant QT prolongation.

HOW IT APPLIES For Dad's LAFB + bradycardia profile, lemborexant's cardiac profile is reasonably reassuring on the conduction axis (unlike Lexapro, which has documented QTc concerns at the elderly maximum dose). This is genuinely one of the better aspects of the lemborexant option if it's pursued later.

TIER 5 — SPECIFIC FINDINGS ★

Takaesu Y, Muraoka H et al. A 12-Week, Open-Label, Multicenter Pilot Study of Lemborexant in Patients With Insomnia Comorbid With Depressive Episodes (SELENADE).

CNS Drugs 2025. Read here.

KEY FINDING 12-week open-label pilot study of lemborexant in patients with comorbid insomnia and depression. Modest HAM-D-17 improvements (~6 points over 12 weeks). Open-label design without control group limits conclusions about direct antidepressant effect.

HOW IT APPLIES The mood improvements observed are likely mediated by sleep improvement rather than direct antidepressant mechanism. Lemborexant shouldn't be framed as treating Dad's depression — that's ketamine's role.